

Pediatric ED/KCH Diversion Criteria

Assumptions:

- Based on current inpatient boarder volumes there is no spillover space for pediatric patients
- KCH has maximized existing inpatient space other than PICU
- Accepting additional patients would exceed available resources (staff or space)
- PEDS ED and KCH Nursing Leadership agree that there are no additional resources
- Divert exceptions:
 - Heart program patients
 - Trauma or surgical emergencies
 - UK Pediatrics hematology/oncology patients
 - Positive newborn screen for metabolic disorders
 - Avoid acceptance of out of state when on divert
- We are not accepting pediatric BH patients from out of state

Process:

- When the criteria below exist the PEDS ED team leader (TL) will collaborate with the PEDS EM attending to discuss flow and need to consider divert.
- Assuming agreement to consider divert the TL will contact the ED nursing administrator on call to affirm moving forward.
- The TL will reach out to the KCH DCN who will notify KCH nursing leadership and the on-call hospitalist of the need for divert.
- KCH Leadership will communicate to ensure no other options exist.
- A huddle will occur to determine if activation of divert is appropriate

Considerations for Peds ED Divert:

ED Volume	Boarders	Expects
<u>>20</u>	>10	>4
>25	> 8	>3
>30	> 6	>2

Coming off of PEDS ED Divert :

- Peds TL will discuss ability to discuss coming off divert when there is PEDS ED capacity and less then 4 inpatient boarders. If in agreement, the PEDS ED TL will notify the ED DCN who will contact the KCH DCN to cancel PEDS ED divert.

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